

Discharge Letter

Dear Dr. Meyer,

Mr. Peter Schulz, born 11/07/1952, presented to our emergency department on 02/20/2024 with acute onset chest pain radiating to the left arm, diaphoresis, and mild dyspnea of approximately 2 hours' duration. His wife drove him in following failure of sublingual nitrate to provide relief. On arrival he was pale, diaphoretic, and in moderate distress. Initial ECG showed ST depression in V4–V6 and lateral leads. Troponin I was elevated at 1.8 ng/mL on first draw, peaking at 4.8 ng/mL at 6 hours. The working diagnosis of NSTEMI was established and the patient was transferred to the catheterization laboratory within 60 minutes.

Primary Diagnosis: Non-ST-elevation myocardial infarction (I21.4)

Coronary Angiography and Intervention: Emergency coronary angiography revealed a 90% stenosis of the proximal left anterior descending artery with TIMI 1 flow. The right coronary artery showed a 50% non-flow-limiting mid-vessel stenosis. Successful PTCA of the LAD with a 3.0 × 18 mm drug-eluting stent; post-intervention TIMI 3 flow achieved. No procedural complications.

Past Medical History: Mr. Schulz has a 15-year history of arterial hypertension managed with amlodipine 10 mg and ramipril 10 mg. Hypercholesterolemia diagnosed 2018, on rosuvastatin 20 mg. Type 2 diabetes diagnosed 2016, on metformin 1000 mg twice daily; most recent HbA1c 7.2%. Ex-smoker, 30 pack-years, cessation 2019. Approximately 8 units of alcohol per week. Appendectomy 1998. No drug allergies.

Physical Examination on Admission: BP 162/96 mmHg, HR 102 bpm, RR 18/min, SpO2 94% on room air improving to 98% on 4 L nasal cannula, temperature 36.8°C. The patient was diaphoretic and in moderate distress. Heart sounds regular with a soft S4 gallop, no murmurs. Lungs clear bilaterally. Abdomen soft, non-tender. No lower limb oedema. Peripheral pulses intact.

Diagnostics: Serial ECGs showed evolving anterior and lateral ST changes.

Echocardiography day 2: anterolateral hypokinesia, overall EF 52%, no significant valvular pathology, no pericardial effusion.

Hospital Course: Dual antiplatelet therapy with aspirin 100 mg and ticagrelor 90 mg twice daily was initiated. Fondaparinux for 48 hours. Bisoprolol uptitrated to 5 mg. Rosuvastatin increased to 40 mg. Metformin held 48 hours post-contrast, restarted day 3. Patient mobilized day 2 and remained haemodynamically stable. No arrhythmias on continuous monitoring.

Medication at Discharge: Aspirin 100 mg once daily (lifelong), ticagrelor 90 mg twice daily (12 months), ramipril 5 mg once daily, bisoprolol 5 mg once daily, rosuvastatin 40 mg once daily, metformin 1000 mg twice daily, pantoprazole 40 mg.

Nursing and Allied Health Summary: Mr. Schulz was educated on the critical importance of dual antiplatelet therapy adherence and the consequences of premature discontinuation, particularly with a newly placed drug-eluting stent. A dedicated cardiac rehabilitation nurse conducted a pre-discharge assessment and provided structured lifestyle modification advice covering dietary changes, graduated physical activity, weight management, and sustained smoking abstinence.

Psychological screening using the Hospital Anxiety and Depression Scale showed mild anxiety. No formal psychiatric referral was deemed necessary. The patient was given written information about the cardiac psychology support service available through the rehabilitation programme. His wife was present for the discharge discussion and confirmed her support for the recommended lifestyle changes.

Secondary Prevention Risk Factor Review: LDL cholesterol 2.8 mmol/L at admission — statin intensification initiated, target LDL below 1.4 mmol/L. Diabetes: HbA1c 7.2%, diabetology review recommended within 3 months. Smoking: ex-smoker since 2019 — cessation maintained. Physical activity: cardiac rehabilitation prescribed. The patient is at high residual cardiovascular risk and is recommended for indefinite follow-up with both cardiology and his general practitioner.

Follow-up: Cardiology outpatient in 4 weeks with repeat ECG. Echo in 3 months. Cardiac rehabilitation first session in 2 weeks. GP within 1 week for blood pressure and glucose assessment. Patient advised not to discontinue any medications without prior discussion with his cardiologist. Driving restriction for 1 week post-procedure explained. Written stent care information provided.

Dr. Andreas Müller, MD

Senior Cardiologist